HOW TO CREATE A CASE CLASSIFICATION SHEET

Professional & Systematic Pharmacovigilance Notes

Topic	Case Classification Sheet creation and practical classification of Individual Case Safety Reports (ICSRs)
Application	Pharmacovigilance case processing, ADR monitoring, drug-safety internships and exam/interview preparation
Important	Exact fields and coding conventions may vary by organization, validated safety database and SOP.

These notes provide a standard practical framework. Always follow the applicable company/SOP, validated safety database procedures and current regulatory requirements for real cases.

1. What is a Case Classification Sheet?

A case classification sheet is a structured working document used by pharmacovigilance personnel to capture and classify key characteristics of an Individual Case Safety Report (ICSR). It converts information from the source report into standardized fields supporting case processing, medical review, coding, seriousness assessment, causality assessment, expectedness and follow-up.

- Provides a standardized snapshot of the case.
- Helps reviewers identify missing information and follow-up requirements.
- Supports consistent classification across cases.
- Creates an auditable working record of processing decisions.
- May be implemented as a controlled spreadsheet or within a validated safety database.

2. Why is Case Classification Important?

- Ensures important safety information is not missed.
- Improves consistency between case processors and reviewers.
- Helps determine whether a report represents a valid case and what processing pathway is required.
- Facilitates prioritization of serious cases and cases requiring expedited attention.
- Supports accurate medical/product coding.
- Helps identify duplicate and follow-up reports.
- Provides a concise hand-off document for medical review and quality control.

3. Basic Workflow

Receive source report

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Validate minimum case information

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Create/identify case number

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Check duplicate status

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Extract patient + event + product information

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Classify seriousness / causality / expectedness as applicable

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Identify coding and follow-up needs

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Quality check

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Enter/update validated safety database according to SOP

4. Recommended Case Classification Sheet Structure

Section	Typical fields	Purpose

Case Identification	Case ID, initial/follow-up, date received, country, source type	Traceability and tracking
Patient	Patient identifier/initials, age, sex, DOB, weight, relevant history	Patient characterization
Event	Verbatim event, onset, outcome, seriousness criteria	Clinical event assessment
Suspect Product	Product/ingredient, strength, dose, route, frequency, indication, therapy dates	Exposure characterization
Concomitant Products	Other medicines/products and relevant dates/indications	Alternative causes/interactions
Classification	Seriousness, causality, expectedness/listedness, special situation flags	Safety/regulatory classification
Coding	MedDRA terms, product coding and other controlled terminology	Standardized database representation
Follow-up / QC	Missing information, follow-up, reviewer comments, QC status	Completeness and quality

Production case processing should use a controlled and appropriately validated system rather than an unsecured personal spreadsheet.

5. Section 1 — Case Identification

- Case ID: unique internal identifier.
- Report type: initial or follow-up, where applicable.
- Date received: date the organization/monitoring centre received the report.
- Country: country of occurrence/report origin as applicable.
- Source: spontaneous, literature, study, patient support, regulatory authority, digital source or other applicable source.
- Source/reference number: allows retrieval of the original report.

6. Section 2 — Patient Information

- Record only information necessary and permitted under privacy/SOP requirements.
- Age/date of birth and sex are important for clinical interpretation.
- Record weight when relevant to dose assessment.
- Capture relevant medical history, allergies, pregnancy or other special factors when documented.
- Never invent missing demographics.

7. Section 3 — Adverse Event Information

- Preserve the reporter's original wording as the verbatim term.
- Describe useful clinical details and chronology.
- Record onset date/time where available.
- Record outcome: recovered/resolved, recovering/resolving, not recovered, recovered with sequelae, fatal or unknown, according to the applicable system.
- Classify seriousness using defined criteria, not merely the word 'severe'.
- Capture relevant laboratory, imaging and diagnostic findings.

8. Section 4 — Suspect Drug/Product Information

- Product and active ingredient.
- Brand/manufacturer where available.
- Strength/formulation where relevant.
- Dose, route and frequency.
- Indication.
- Start/stop dates.
- Batch/lot and expiry where relevant and available.
- Action taken, according to the applicable database/form.

9. Section 5 — Concomitant Products

- List relevant medicines/products during the exposure period.
- Capture dose, route, frequency, indication and dates when available.
- Use concomitant therapy information when considering alternative causes and interactions.

10. Section 6 — Seriousness Classification

- Death.
- Life-threatening.
- Initial or prolonged hospitalization.
- Persistent/significant disability or incapacity.
- Congenital anomaly/birth defect.
- Other medically important event.

11. Section 7 — Causality Classification

- Use the framework required by the organization/regulator.
- PvPI uses the WHO–UMC causality assessment framework.
- Consider temporal relationship, alternative explanations, dechallenge, rechallenge when it actually occurred, and clinical/pharmacological plausibility.
- Temporal association alone does not prove causality.

12. Section 8 — Expectedness / Listedness

- Assess whether the nature and specificity of the reaction are consistent with the applicable reference safety information.
- Use the correct reference document for the case context.
- Document the reference when required.
- Expectedness is distinct from seriousness and causality.

13. Section 9 — MedDRA Coding

MedDRA is a standardized medical terminology used in regulated safety information. The classification sheet can capture the selected MedDRA term(s) and coding rationale where required.

- Preserve reporter wording before coding.
- Select the most appropriate term supported by the documented clinical concept.
- Do not add unsupported diagnoses.
- Use the current MedDRA version and organization-specific conventions.
- Follow the coding SOP and medical review process.

Coding mindset: Reporter wording → understand clinical concept → select term → verify hierarchy/version → document clarification if needed → QC.

14. Section 10 — Special Situation Flags

Depending on product and workflow, cases may require identification of special situations such as:

- Pregnancy/lactation exposure
- Medication error
- Overdose
- Off-label use
- Misuse
- Abuse
- Occupational exposure
- Lack of efficacy
- Product quality issue
- Drug interaction
- Special population use

A special situation is not necessarily an adverse event. Record the special situation separately from any associated adverse event and follow applicable reporting rules.

15. Duplicate Case Check

- Compare patient characteristics.
- Compare suspect product and exposure dates.
- Compare event terms and onset dates.
- Compare source/reporter information.
- Compare narrative and clinical details.
- If potentially duplicate, follow the organization's duplicate-management procedure rather than creating an uncontrolled second case.

16. Follow-up Assessment

- Identify clinically important missing information.
- Ask focused follow-up questions.
- Prioritize information that could change validity, seriousness, causality, outcome, product identification or reporting decisions.
- Record follow-up status/date according to SOP.
- Preserve the original reporter information and maintain traceability.

17. Sample Case Classification Sheet

Field	Example
Case ID	PV-2026-0001
Report type	Initial
Source	Healthcare professional
Patient	Initials A.B.; 45 years; female
Suspect drug	Drug X 500 mg orally twice daily
Indication	Bacterial infection
Therapy dates	01-Oct-2026 to 04-Oct-2026
Event	Generalized maculopapular rash
Onset	03-Oct-2026
Seriousness	Non-serious, if no seriousness criterion is present
Action taken	Drug withdrawn
Outcome	Recovering
Causality	Probable/Likely if full case evidence supports it
Concomitant drugs	None reported
Follow-up	Confirm final outcome and relevant clinical findings

Fictional training example only; do not copy into a real safety report.

18. Recommended Spreadsheet Columns

A practical training sheet can include:

Case ID | Initial/Follow-up | Date Received | Country | Source | Patient Age | Sex | Suspect Product | Active Ingredient | Dose | Route | Frequency | Indication | Start Date | Stop Date | Verbatim Event | Onset Date | Outcome | Serious? | Seriousness Criterion | Causality | Expectedness/Listedness | MedDRA PT | Special Situation | Concomitant Drugs | Duplicate Status | Follow-up Required | QC Status | Reviewer Comments

19. Quality Control Checklist

- ☐ Correct case ID and report type
- ☐ Source and date received documented
- ☐ Patient information complete and privacy-appropriate
- ☐ Event description and onset reviewed
- ☐ Outcome recorded
- ☐ Seriousness criterion correctly classified
- ☐ Suspect product identified
- ☐ Dose/route/frequency and therapy dates reviewed
- ☐ Concomitant products assessed
- ☐ Causality completed according to SOP
- ☐ Expectedness/listedness assessed against correct reference
- ☐ MedDRA coding reviewed using current version
- ☐ Special situation flags checked
- ☐ Duplicate check completed
- ☐ Follow-up needs identified
- ☐ Reviewer/QC status documented

20. Common Mistakes

- Creating an uncontrolled sheet without SOP oversight.
- Mixing source facts with assumptions.
- Calling an event serious because it sounds severe.
- Assigning causality solely because the event followed drug exposure.
- Failing to check duplicates.
- Using outdated terminology or inconsistent coding.
- Adding a diagnosis not supported by the source.
- Failing to identify follow-up-critical missing data.
- Storing identifiable patient information in an unsecured spreadsheet.
- Changing source information without traceability/audit trail.

21. Causality, Severity, Seriousness & Expectedness

Term	Meaning
Causality	Likelihood that the medicine is related to the event.
Severity	Intensity of the event, e.g. mild/moderate/severe.
Seriousness	Regulatory classification based on defined outcomes/criteria.
Expectedness	Whether the reaction is consistent with applicable reference safety information.

22. Exam / Interview Questions

Q. What is a case classification sheet?

A. A structured working document used to capture and classify key information from an ICSR.

Q. Why is it useful?

A. It supports standardization, completeness, review, follow-up, coding and quality control.

Q. What are its major sections?

A. Case identification, patient, event, suspect product, concomitant products, classification, coding, special situations and follow-up/QC.

Q. Is seriousness the same as severity?

A. No. Seriousness is based on defined regulatory criteria; severity describes intensity.

Q. What should be considered for causality?

A. Temporal relationship, alternative causes, dechallenge, rechallenge when it occurred, and clinical/pharmacological plausibility.

Q. Why perform duplicate checks?

A. To prevent the same safety case from being processed as separate cases.

Q. Why preserve verbatim terms?

A. They represent the original reporter information and support transparent coding.

Q. What is MedDRA used for?

A. Standardized coding of medical concepts in safety information.

Q. What should happen when information is missing?

A. Do not guess; identify the gap and determine whether follow-up is needed.

23. Quick Revision Algorithm

RECEIVE REPORT

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VALIDATE CASE

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ASSIGN / IDENTIFY CASE

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DUPLICATE CHECK

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PATIENT + EVENT + PRODUCT

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SERIOUSNESS

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CAUSALITY

↓

EXPECTEDNESS / LISTEDNESS

↓

MEDDRA + PRODUCT CODING

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SPECIAL SITUATIONS

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FOLLOW-UP

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QUALITY CONTROL

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FINAL DATABASE / REGULATORY PROCESSING

24. Key Takeaways

- A classification sheet is a structured pharmacovigilance working tool.
- Build it around traceability, patient/event/product information and regulatory classification.
- Separate reporter facts from processor conclusions.
- Seriousness, severity, causality and expectedness are distinct.
- Duplicate checking is essential.
- MedDRA coding must follow the current version and organizational conventions.
- Missing information should trigger a documented follow-up decision, not guessing.
- Real-world processing must follow the applicable SOP, regulatory requirements and data-privacy controls.

25. References

1. WHO / Uppsala Monitoring Centre — WHO–UMC causality assessment framework.

2. Indian Pharmacopoeia Commission (IPC), Pharmacovigilance Programme of India (PvPI) — ADR reporting and pharmacovigilance resources.

3. ICH E2A — Clinical Safety Data Management: Definitions and Standards for Expedited Reporting.

4. ICH E2B(R3) — Individual Case Safety Reports data elements and standards.

5. MedDRA MSSO — MedDRA Introductory Guide and current terminology documentation.

Video topic requested: “How to Create Case Classification Sheet” — YouTube video ID LHbUuZqadys.

Exact columns, coding conventions, QC checkpoints and reporting timelines may vary by organization and jurisdiction; use the current controlled SOP for actual case processing.